



Armadale Kalamunda Group

Education – Childbirth and Beyond Guideline

1. Purpose

- Structured antenatal education that is suited to the individual can help women to be informed about pregnancy, birth and parenting. Psychological preparation for parenthood may have benefits for parents' mental health, parenting and infant development¹.
- To offer an education programme to expectant parent's that will increase their knowledge of the physiological and psychological events of pregnancy, childbirth and parenting.
- To promote staff and student's awareness in the benefits of childbirth and parenting education courses and facilitating their participation in providing evidence-based activities to meet these needs.
- Ensure that healthcare staff collaborate in a multidisciplinary approach to optimise quality of parent education delivered.
- To ensure that pregnant people and their partners attending Armadale Health Service receive appropriate information regarding the benefits and management of breastfeeding.

2. Applicability

This guideline applies to the midwifery discipline within the obstetrics department of Armadale Health Service.

In person & Online Delivery of Classes

In response to the SARS-CoV-2 pandemic, Armadale Health Service began delivery of classes in an online format. The midwife and physiotherapist deliver education via group telehealth consultations. Due to the positive feedback this has continued however In person classes have also been reintroduced as requested by clientele with positive results.

Guidelines

2.1 Key points

- Couples should have completed classes prior to 36 weeks of their pregnancy
- Consistency of information is very important especially if it is provided by different professionals
- Information given to couples must be non-biased and evidence based.
- Multidisciplinary parent education is associated with lower levels of anxiety, larger number of vaginal deliveries and shorter hospitalisation of the newborn.

Before referencing this guideline document please ensure that you have the latest version from the [Policy Library](#) information hub page.

- Research suggests that parent education programmes have a potential role to play in the promotion of mental health.
- Delivery of breastfeeding education to the expectant parents lengthens the overall duration of breastfeeding.
- Partners are equally as important in the delivery of childbirth and parenting education, as research suggests that partners of the baby have now become the primary support for most birthing people.

2.2 Themes for programme content include the following

- Physical wellbeing (nutrition, physical activity, smoking, alcohol, oral health)
- Changes and discomforts related to the physiological changes of pregnancy
- When to seek advice if there are concerns regarding the pregnancy and labour
- Labour (stages of labour, positions, breathing and relaxation, comfort measures and support, pain relief)
- Birth and the immediate postnatal period
- Common and uncommon interventions and outcomes in pregnancy and labour
- Early parenting skills, feeding the baby and newborn behaviours
- Emotional wellbeing and mental health during pregnancy and after the baby is born (maternal-fetal attachment, adapting to change, expectations, coping skills, knowing when to get help)
- Services and supports in the community

3. Roles and responsibilities

Role	Responsibility
Registered Midwife	Provides antenatal education Accesses client list prior to class and emails links to participants to join online/in person class prior to commencement of first class Work collaboratively in session with Physiotherapist Emails evaluation forms post session and provide further links on requested information/topics Collates feedback for reporting to Midwifery Management
Physiotherapist	Works collaboratively with the midwife to run the class on discomforts of pregnancy and positions and comfort measures for labour

4. Supporting information

The following documents support the implementation of this policy:

- [Clinical Practice Guidelines: Pregnancy Care](#) - Module 9. Australian Government Department of Health and Ageing
- [Pregnancy care: My Baby WA Pregnancy App](#). Health consumers Council
- [Having a Baby in WA](#). Department of Health Western Australia

5. Compliance, monitoring and evaluation

Compliance to guideline will be reviewed by routine collection of feedback requested from attendees of each education session.

Annual review of topics to ensure education aligns with current standards.

6. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 2 – Action 2.6: The health service organisation has processes for clinicians to partner with patients and/or their substitute decision-maker to plan, communicate, set goals, and make decisions about their current and future care
- Standard 2 – Action 2.7: The health service organisation supports the workforce to form partnerships with patients and carers so that patients can be actively involved in their own care
- Standard 2 – Action 2.10: The health service organisation supports clinicians to communicate with patients, carers, families and consumers about health and health care so that:
 - a. Information is provided in a way that meets the needs of patients, carers, families and consumers
 - b. Information provided is easy to understand and use
 - c. The clinical needs of patients are addressed while they are in the health service organisation
 - d. Information needs for ongoing care are provided on discharge

7. References

1. Australian Government: Department of Health, Clinical Practice Guidelines: Pregnancy Care – Module 9. Australian Government Department of Health and Ageing, Canberra. (2020)
<https://www.health.gov.au/resources/publications/pregnancy-care-guidelines>

8. Document owner

Midwifery Unit Manager Obstetrics

Enquiries relating to this guideline may be directed to:

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9. Review

This guideline will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V4	27/06/2025	27/05/2028	Applicability – Both online and in person classes

			Minor administration amendments
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10. Authorisation

This guideline has been authorised and issued by the Director Nursing and Midwifery.

Authorised by	Jane Husain – A/Director Nursing and Midwifery
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